



Goals of Patient Care Policy

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	All services	Medical, Nursing and Allied Health

2. Purpose

Goals of Patient Care (GoPC) is a clinical framework which prompts and facilitates proactive shared decision making between the clinicians and patients and/or person responsible / family / carer(s) to ensure treatment provided is aligned to the patient's preferences, needs, values and wishes.

The purpose of this policy is to support clinicians with the implementation of the GoPC process, including the completion of the relevant associated documentation.

Goals of Patient Care Form can be completed either on paper or using electronic form. A state-wide care planning electronic form (eForm) can assist clinicians in the Goals of Patient Care process and is the preferred option.

If there is no documented evidence of a completed GoPC form:

- The default is for full cardiopulmonary resuscitation (CPR) to occur
- If a patient is thought to be unlikely to survive CPR by senior clinicians, then the treatment does not have to be offered on the grounds of harm rather than benefit.

3. Policy statement

- The GoPC form should be completed as early as possible in the patient journey.
- For emergency admissions the GoPC form should be completed within 48 hours of admission for patients (where appropriate). However, if clinical deterioration is likely or urgent interventions are planned then the GoPC form should be completed at the earliest opportunity.
- The GoPC process should be implemented as a matter of priority to guide the clinical care of patients with advanced, life-limiting conditions. It is at the discretion of the treating clinician to use this form in other groups of patients.
- In patients with advanced health directives (AHD) the contents of the AHD should be used to populate the GoPC form. The AHD should be available for inspection in the medical records. In certain cases, the AHD may no longer represent a medically appropriate management strategy. In these scenarios multi-disciplinary consensus should be sought and a GoPC form should be completed to reflect the most appropriate medical management.

- The GoPC form is valid for the current admission but may be extended for a period of up to 12 months with appropriate Consultant endorsement.
- In certain scenarios where, new clinical information becomes available the GoPC may need to be altered. In these cases, the doctors should cross out the old form, date, sign, and print name but leave it in the medical record. A new form should be completed. The doctor must make an entry in the clinical notes explaining the rationale for altering the GoPC form. The reasons for the change to the GoPC form should be documented in the medical records.
- No completed GoPC eform should be written on following printing. If a GoPC requires amendment a **NEW** eform must be completed. Any GoPC copies that are written on and sent to the medical record department for scanning will be sent back to the ward of origin un-scanned requesting that a **NEW** eform is completed.

4. Priority setting:

To support the identification of patients who will require a GoPC discussion as a matter of priority, the following criteria has been established below:

- Patients that meet the Supportive and Palliative Care Indicators Tool (SPICT) criteria should have a GoPC form completed
- Any patient who is anticipated to clinically deteriorate
- Patients over the age of 70 years
- Patients with advanced, life-limiting conditions
- Patients with Advanced Health Directives (AHD).

For patient's incapable of making an informed decision for GoPC, the discussion is to be held with their family/carer/delegated authority – refer to the WA Consent to Treatment Policy for further details.

5. Accountability and responsibilities:

5.1 Medical staff (Registrar level or above)

- Review the DMR for any previous GoPC via the “Not for CPR” filter on the left side of the front page of the DMR.

There is no prompt in the DMR to indicate that a current GoPC form exists. Staff must check for the current form behind the “CPR?” filter

- Responsible for identifying patients for GoPC discussion (see inclusion criteria).
- Responsible for exploring the patient's concerns about their end-of-life preferences/care. For Aboriginal patients it is recommended that the assistance of an Aboriginal Liaison Officer be enlisted to ensure that goals are formulated in accordance with the patient's cultural values.
- Responsible for providing the patient with information regarding any prognosis and the likely response to cardiopulmonary resuscitation (CPR) to inform the patient, their carer and/or families decision with regards to goals of care.
- Responsible for completing the GoPC form in a timely manner:
 - Early in the admission for patients who have deteriorated or who are likely to deteriorate

- Early in the course of advanced or life limiting illness
- Where an Advanced Health Directive (AHD) or Advanced Care Plan (ACP) exists.
- Responsible for ensuring that all relevant information has been captured following discussion with the patient, their carer and/or family.
- Responsible for reviewing and updating any goals of patient care in response to changing clinical and/or patient, carer and/or family preference.
- Responsible for ensuring they are aware of patients' treatment preferences as outlined in the GoPC form and complying to them at all times (within the current admission or time period for which the form is valid).

5.1.1 Members of the multidisciplinary team

- Responsible for identifying patients for GoPC discussion (see inclusion criteria).
- Allied Health, nursing, and other members of the multidisciplinary team (MDT) are responsible for contributing to the process and providing input into the decisions outlined in the GoPC form as necessary.
- Responsible for initiating discussion about completing the GoPC process with medical members of the treating team if an applicable patient presents to the ward and GoPC has not been considered.
- Responsible for ensuring they are aware of patients' treatment preferences as outlined in the GoPC form and complying to them at all times (within the current admission or time period for which the form is valid).

6. Roles and responsibilities

[This section describes the allocation of responsibility for implementation of the Policy. Specify any unique responsibilities which are assigned to various individuals. Any function or authority assigned to a staff member must be within their area of accountability.]

Role	Responsibility
Medical staff (registrar level or above)	● Identify patients for discussion ● Explore care preferences ● Provide clear information on condition ● Complete the GoPC form ● Update form in response to changing preferences ● Comply with patients wishes
Member of the multidisciplinary team	● Identify patients for discussion and communicate to medical staff ● Provide input into the decision ● Review patient preferences regularly in case of changes made ● Comply with patients wishes

7. Legislative context

The following documents are required to give effect to this policy:

- Health Department of WA (HDWA) [Recognising and Responding to Acute Deterioration Policy](#)
- HDWA [Health Clinical Handover Policy MP 0095](#)
- HDWA [Health Consent for Treatment Policy MP 0175](#)

8. Supporting information

The following documents support the implementation of this policy:

- [Goals of Patient Care eForm Procedure](#)
- [End of Life Care Procedure](#)
- [Recognition and Response Acute Deterioration Policy](#)
- [Advance Health Directive and Enduring Power of Guardianship Guideline](#)

9. Compliance, monitoring and evaluation

The AKG Comprehensive Care Steering Committee will be accountable for monitoring compliance with this policy, via the following GoPC policy compliance audit including (annual):

- Number of patients with a GoPC form indicated at the time of admission
- Number of patients who have a valid GoPC form available within 48 hrs of admission
- Number of patients who have a valid GoPC form prior to a MET call activation
- Number of patients that have had the GoPC form reviewed/completed post MET call
- Documented decision rationale for agreed GoPC
- Documented Consultant endorsement
- Other health directives available:
 - Advance Health Directive (AHD)
 - Advance Care Plan (ACP)
 - My Health Record uploaded: Patient Consent

Combined Brief Risk Assessment (CoBRA) auditing of GoPC including (ongoing):

- GoPC completed within 8 hrs of admission
- GoPC form in the bedside record

GoPC indicated

10. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.4 The health service organisations ensures that its informed consent processes comply with legislation and best practice:
- Standard 2 – Action 2.5 The health service organisation has processes to identify:
 - a. The capacity of a patient to make decisions about their own care
 - b. A substitute decision-maker if a patient does not have the capacity to make decisions for themselves
- Standard 2 – Action 2.6: The health service organisation has processes for clinicians to partner with patients and/or their substitute decision-maker to plan, communicate, set goals, and make decisions about their current and future care

- Standard 5 – Action 5.3: Clinicians use organisational processes from the Partnering with Consumers Standard when providing comprehensive care to:
 - a. Actively involve patients in their own care
 - b. Meet the patient's information needs
 - c. Share decision-making
- Standard 5 – Action 5.13: Clinicians use processes for shared decision making to develop and document a comprehensive and individualised plan that:
 - a. Addresses the significance and complexity of the patient's health issues and risks of harm
 - b. Identifies agreed goals and actions for the patient's treatment and care
 - c. Identifies the support people a patient wants involved in communications and decision-making about their care
 - d. Commences discharge planning at the beginning of the episode of care
 - e. Includes a plan for referral to follow-up services, if appropriate and available
 - f. Is consistent with best practice and evidence
- Standard 5.20 – Action 5.20: Clinicians support patients, carers and families to make shared decisions about end-of-life care in accordance with the National Consensus Statement: Essential elements for safe and high-quality end-of-life care
- Standard 6 – Action 6.3: Clinicians use organisational processes from the Partnering with Consumers Standard to effectively communicate with patients, carers and families during high-risk situations to:
 - a. Actively involve patients in their own care
 - b. Meet the patient's information needs
 - c. Share decision-making
- Standard 6 – Action 6.9: Clinicians and multidisciplinary teams use clinical communication processes to effectively communicate critical information, alerts and risks, in a timely way, when they emerge or change to:
 - a. Clinicians who can make decisions about care
 - b. Patients, carers and families, in accordance with the wishes of the patient
- Standard 6 – Action 6.10: The health service organisation ensures that there are communication processes for patients, carers and families to directly communicate critical information and risks about care to clinicians
- Standard 6 – Action 6.11: The health service organisation has processes to contemporaneously document information in the healthcare record, including:
 - a. Critical information, alerts and risks
 - b. Reassessment processes and outcomes
 - c. Changes to the care plan
- Standard 8 – Action 8.3: Clinicians use organisational processes from the Partnering with Consumers Standard when recognising and responding to acute deterioration to:
 - a. Actively involve patients in their own care
 - b. Meet the patient's information needs
 - c. Share decision-making

11. Policy owner

Director of Clinical Services

Enquiries relating to this policy may be directed to:

Title: Deputy Director Clinical Services
Department: Executive Team
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12. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
3	18/06/2024	18/06/2027	

13. Authorisation

This policy has been authorised and issued by the Executive Director.

Authorised by	A/Director of Clinical Services – Tania Strickland
Authorisation date	18/06/2024
Published date	19/06/2024